coursera

Anxiety and Related Disorders

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

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Introduction to Psychological Disorders

Module 3
Opening to Anxiety Disorders

Module 4
Components of Anxiety

Module 5
Disorders that Involve Anxiety Part 1

Module 6
Disorders that Involve Anxiety Part 2

Social Anxiety Disorder
- ✓ Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Social Anxiety Disorder
  Reading • 15 min
- ✓ Check Your Understanding: Social Anxiety Disorder
  Practice Assignment • Grade: 100%

Obsessive-Compulsive Disorder
- ✓ **Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Obsessive-Compulsive Disorder**
  Reading • 15 min
- ✓ The Vicious Cycle of OCD
  Video • 2 min
- ○ Check Your Understanding: Obsessive-Compulsive Disorder
  Practice Assignment • 4 min

Treatment for Disorders that Involve Anxiety
- ○ Types of Treatment for Disorders that Involve Anxiety
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Key Takeaways
- ○ Key Takeaways: Disorders that Involve Anxiety Part 2
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Module 6 Quiz
- ○ Module Quiz: Disorders that Involve Anxiety Part 2
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Conclusion

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Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Obsessive-Compulsive Disorder

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Diagnostic Criteria

Of all the disorders that involve anxiety, you may be most familiar with **obsessive-compulsive disorder (OCD)**, a disorder that involves recurrent thoughts (obsessions) or behaviors (compulsions). Though familiar with obsessive compulsive disorder through media and pop culture, many people misunderstand or minimize the severity of the disorder.

People suffering from obsessive compulsive disorder experience obsessions and compulsions. **Obsessions** as recurrent, unwanted, and intrusive thoughts that lead to anxiety and distress (APA, 2013). Common obsessions include the fear of being contaminated by dirt or germs, the desire to have order and symmetry, superstitions, and thoughts that are violent or sexual.

Compulsions are ritualized, repetitive behaviors, often undertaken in response to the anxiety caused by obsessions (APA, 2013). Common behaviors can include frequent handwashing or checking behavior, such as checking to make sure doors are locked, the stove is turned off, or loved ones are safe. Compulsions also include "mental acts," such as counting, tapping, or repeating a phrase.

As is true with other disorders that cause anxiety, it is not the thoughts or behaviors that are the main problem. Everyone washes their hands, more frequently now since the COVID-19 pandemic, and checking behaviors are not atypical. It is the frequency or amount of time spent engaging in thoughts and behaviors that make them obsessions and compulsions, respectively. For a person to be diagnosed with obsessive compulsive disorder, they must spend at least an hour a day engaged in obsessive thinking and/or compulsive behavior and these actions must cause distress, and impair the person's functioning in some way (in relationships, at school or work; APA, 2013).

Consider the story of Shane Larkin, a professional basketball player who played for a time with the Boston Celtics. Sports writer Jackie MacMullan chronicled Larkin's struggle with obsessive compulsive disorder (2018, August 22):

> “
>
> **SHANE LARKIN OPENS** his eyes, sits up and embarks on his own tortured version of "Groundhog Day." He grabs the remote, clicks on SportsCenter and hops out of bed to wait for his "number." He is 8 years old, and every morning presents a new set of unpredictable parameters that are purely arbitrary. As he starts to get dressed for school—a ritual that can last a few minutes or sometimes hours, depending on the number for the day—he notices an image of Ray Allen flickering on his television screen. Allen, it seems, hit eight 3-pointers in a game the night before. Suddenly, a sensory message makes a beeline for Shane's brain and informs him of the number for the day: eight.
>
> "And then I know," Larkin tells ESPN, "that I have to wash my hands eight times."
>
> ”

For the rest of that day, any time Larkin touched anything that made him anxious, he would have to return to the washroom and wash his hands eight times. On days when his number for the day was smaller, he was grateful he would not have to spend quite so much time handwashing.

Common obsessions include worrying about harming yourself or others, being contaminated by dirt or germs, concerns about following superstitions, and fearing you have forgotten something important. Common compulsions that people undertake to combat such obsessions include washing hands, ordering and arranging things, praying excessively, hoarding newspapers and empty containers, and checking behavior, such as repeatedly checking to be sure the stove is turned off or the doors are locked

Patients with obsessive compulsive disorder vary in how much insight they have into the beliefs associated with their disorder (APA, 2013). Patients with good insight know the house will probably not burn down if the stove is not checked 30 times. At the other end of the continuum are patients with poor insight/delusional beliefs who are convinced the house will burn down if the stove is not checked. Insight can vary within an individual over the course of the illness. Patients with poor insight have worse long-term outcomes than patients with good insight.

Compulsive behavior provides short-term relief. Eventually, though, the obsessive thoughts will return, requiring additional compulsive behavior. Compulsive behavior is maladaptive. Repeated checking behavior may cause a person to miss work or school or not have enough time to study for exams or complete assignments, increasing the likelihood the person will be fired from work or fail at school. Excessively seeking reassurance from others damages relationships, reducing opportunities for social support that could help a person cope with their distress.

Prevalence

The lifetime prevalence of OCD is about 1.5% (Kessler et al., 2005). Prevalence of OCD is lowest for those over the age of 60 (about 0.5 %) and highest for those in the 30-44 age group (about 2.5%), followed by the 18-29 age group (about 2%).

In childhood more boys than girls are diagnosed with OCD, but in adulthood, more women than men receive a diagnosis of OCD. There are similar rates of OCD, as well as course and prognosis, around the world, although there are a few cultural differences in the types of obsessions and compulsions (APA, 2013).

Contributing Factors

Biological Factors in Developing OCD

Biological contributors include genetic factors, atypical functioning in brain circuitry and brain structures involved in executive function and regulation of emotion, and activity of the neurotransmitters serotonin and dopamine in the brain.

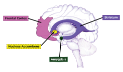
Genetics

Studies conducted as early as the 1930s have found consistent evidence that some forms of obsessive compulsive disorder run in families (Grabe et al, 2006; Hettema et al., 2001; Pauls, 2010; Pauls et al., 2014). For example, if one member of a twin pair has obsessive compulsive disorder, the other twin is at higher risk for developing the disorder. This is especially true for identical twins, who share 100% of their genes. If one identical twin has obsessive compulsive disorder, the probability that the other twin develops obsessive compulsive disorder can be as high as 80% (Hudziak et al., 2004; Inouye, 1965). For fraternal twins, who share 50% of their genes, if one twin has obsessive compulsive disorder there is a 25%–50% chance of the other twin developing the disorder (Hudziak et al., 2004; Inouye, 1965). When considering possible influences on the likelihood of developing symptoms of obsessive compulsive disorder, Shelley Taylor (2011) found that genetics accounted for about 40% of the variability in developing symptoms. Unique environmental influences, such as a stressful life event experienced by only one twin, explained about 50% of the variability in the symptoms of obsessive compulsive disorder (Taylor, 2011).

Brain Structure and Function

A number of studies have provided evidence that brain circuitry involving neural pathways from regions of the frontal lobe to the striatum, called the frontostriatal circuit, along with an area of the brain called the nucleus accumbens are implicated in obsessive compulsive disorder (Saxena & Rauch, 2000; Milad & Rauch, 2012; Pauls et al., 2014). The frontostriatal circuit is involved in tasks of cognitive flexibility and behavioral adaptation, and the nucleus accumbens influences feelings of reward and satisfaction (Chamberlain et al., 2006; Salgado & Kaplitt, 2015). Typically individuals are able to control the common experience of intrusive thoughts, urges, and behaviors. Individuals with obsessive compulsive disorder, however, have difficulty pushing obsessive thoughts and feelings out of their minds and inhibiting compulsive behaviors. Researchers believe that overactivity in the neural pathways of the frontostriatal circuit impairs the kind of executive function needed to control intrusive thoughts and inhibit compulsive behavior (Milad & Rauch, 2012; Pauls et al., 2014, but see Abramovitch et al., 2013). The image Frontostriatal Circuitry and OCD shows a representation of these areas of the brain.

Frontostriatal Circuitry and OCD



Note. The brain's frontostriatal circuit includes the frontal lobe and the striatum. Overactivity in these brain areas, along with the nucleus accumbens and the amygdala, contribute to obsessive compulsive disorder.

Researchers have also found evidence supporting the role of two additional brain regions: the anterior cingulate cortex (ACC), which is involved in error monitoring, and the amygdala, which plays a role in regulation of fear (Milad & Rauch, 2012; Rauch et al., 2003; Shin & Liberzon, 2010). Productive human behavior requires a person to have the ability to identify appropriate behavior in a situation, and easily adapt when an error is made (Orr & Hester, 2012). Patients suffering from obsessive compulsive disorder show hyperactivation of the ACC, which may account for difficulties in adjusting and controlling their behaviors (Saxena & Rauch, 2000). Further, obsessive compulsive disorder patients show exaggerated amygdala activity, resulting in fear responses to stimuli that would not produce a fear response in people who do not have obsessive compulsive disorder (Milad & Rauch, 2012).

Neurotransmitters

Neurotransmitters are chemicals released by neurons that carry messages to other neurons. The neurotransmitter that has most consistently been implicated in obsessive compulsive disorder is serotonin (Bandelow et al., 2016). In the brain, serotonin is an excitatory neurotransmitter that affects mood; that is, when serotonin is released by neurons it makes it more likely that other neurons will fire. When more serotonin is present, mood is elevated. Its importance in obsessive compulsive disorder, though, has more to do with its role in the frontostriatal circuit. As explained previously, dysfunctions of the frontostriatal system trigger obsessive compulsive disorder symptoms. These problems can be thought of as difficulties in the ability of the frontal lobe to communicate with other parts of the brain, a process that is influenced by levels of serotonin (Murphy et al., 1990). Treating individuals with obsessive compulsive disorder with medications that increase serotonin levels appears to reduce overactivity in the frontostriatal circuit (Lissemore et al., 2018). The neurotransmitter dopamine, another important chemical messenger in the frontostriatal circuit, also appears to play a role in the experience of obsessive compulsive disorder symptoms (Pauls et al., 2014).

Environmental Factors Influencing OCD

Cognitive Contributors

Clinical psychologists Dianne Hezel and Richard McNally (2016) reviewed studies examining the dysfunctional beliefs that play a role in obsessive compulsive disorder. They identified three cognitive distortions that perpetuate the obsessions and compulsions experienced by patients: overestimation of threats and a sense of inflated responsibility, a need to control thoughts, and intolerance of uncertainty.

- **Overestimation of Threats and Inflated Responsibility:** Clinical psychologist Paul Salkovskis (1985) and many others believe that at some point, everyone experiences the same kind of intrusive thoughts that patients with obsessive compulsive disorder experience. People with obsessive compulsive disorder, though, seem to misinterpret the importance of these thoughts, which then leads them to engage in compulsions to relieve the anxiety produced by the intrusive thoughts (Rachman, 1997). Salkovskis developed a cognitive behavioral model of obsessive compulsive disorder, in which he proposed that individuals with obsessive compulsive disorder catastrophically misinterpret the importance of normal, distressing thoughts (Hezel & McNally, 2016, p. 221).
  For example, such individuals may misinterpret intrusive thoughts as indications of potential harm or danger and feel responsible for preventing the harm. A person without obsessive compulsive disorder may think food being served at a restaurant could easily be poisoned, but would likely dismiss the thought as silly. A patient with obsessive compulsive disorder, however, may believe the food is poisoned and believe they and the other diners in the restaurant are in danger. Thus, the obsessive compulsive disorder patient might speak with the manager about the restaurant's safety record. Even a patient with good insight, who knows at some level their beliefs are irrational, may feel anxious and engage in compulsive behaviors in an effort to manage the anxiety caused by their intrusive thoughts (Hezel & McNally, 2016).
- **Need to Control Thoughts:** Some patients with obsessive compulsive disorder hold the inaccurate belief that their thoughts are powerful and need to be controlled. Specifically, they confuse thought and action (Berle & Starcevic, 2005). They may believe, for example, that thinking something bad is equivalent to doing something bad. That is, a thought about hurting someone is the same as actually hurting them. These patients may also believe that thinking about a negative event makes it more likely that the event will occur (Rachman & Shafran, 1999). The over-importance placed on thoughts, then, is the result of believing that simply thinking about an action is equivalent to engaging in that action and that thinking about an event makes that outcome more likely. When the thoughts are about negative or harmful outcomes, as they typically are in obsessive compulsive disorder, patients feel compelled to control their thoughts to prevent negative outcomes for themselves and others (Shafran & Rachman, 2004).
- **Intolerance of Uncertainty:** People suffering from obsessive compulsive disorder tend to have a low tolerance for uncertainty, finding it distressing (Holaway et al., 2006). The uncertainty may lead to specific compulsions aimed at reducing the anxiety caused by the uncertainty. For example, checking behavior is particularly relevant to feelings of uncertainty because it allows the patient to temporarily reduce the uncertainty they are experiencing (Tolin et al., 2003).

Behavioral Contributors

As mentioned previously, patients with obsessive compulsive disorder have intrusive thoughts that cause them distress and they engage in compulsive behaviors to reduce the distress caused by such thoughts. Though the compulsive behaviors bring only temporary relief from distress, the relief is rewarding. Each time a compulsion provides relief from distress and worry, however temporary, it is being reinforced, which makes it more likely to occur in the future (Taylor, 2005). In operant conditioning terms, compulsions benefit the individual through negative reinforcement. Remember that negative reinforcement involves taking an unpleasant stimulus away to provide relief. In this case, a person with obsessive compulsive disorder experiences distress produced by the obsession, engages in compulsive behavior that relieves the distress, and the relief provides a reward for engaging in the compulsion (Abramowitz et al., 2009).

Avoiding distressing situations altogether is a second way in which behavioral contributors maintain obsessive compulsive disorder symptoms. obsessive compulsive disorder patients who experience distress in certain situations may avoid those situations as a means of avoiding the distress they feel there. Individuals who fear dirt and germs may feel that public transportation is dirty and walk miles to get to work each day, or choose jobs that allow them to work from home. Avoiding the distress caused by using public transportation becomes rewarding, increasing the likelihood that they will continue to walk to work.

The Effects of Racial Discrimination

Discrimination is defined as unfair treatment resulting from membership in a social category such as race, ethnicity, age, gender, gender identity, sexual orientation, or religion. Many research studies have found that discrimination of any kind has a negative effect on the mental health of those who are discriminated against, often affecting generations (Berger & Sarnyai, 2015; Himle et al., 2008; Mouzon et al., 2017). Psychologist Monnica Williams and her colleagues (2012, 2016, 2017) examined how racial discrimination affects the expression of obsessive compulsive disorder in Black people.

The researchers conducted interviews with 3570 people as part of the National Survey of American Life: Coping with Stress in the 21st Century (Himle et al., 2008). Respondents were asked about the presence of obsessions and compulsions and also about their experiences with two types of everyday discrimination: everyday racial discrimination and everyday non-racial discrimination (i.e. discrimination due to age, weight, gender, or other characteristics).

Respondents were first asked how often they had experienced everyday discrimination, including being treated with disrespect, receiving poor restaurant service, being perceived as dishonest, being insulted, and being followed in stores. Respondents who indicated that they had experienced everyday discrimination were then asked if the discrimination was due to their race, gender, sexual orientation, income, age, height, or weight. From the answers to these questions, the researchers divided the discrimination into two categories: everyday racial discrimination and everyday non-racial discrimination.

African Americans who reported experiencing more racial discrimination were more likely to engage in obsessive thinking (Sternthal et al., 2011). Those who experienced non-racial discrimination, however, were not. The same pattern was observed for compulsions: African Americans who experienced high levels of racial discrimination were more likely to perform compulsive behavior, including handwashing, checking behavior, and arranging behavior while those who experienced non-racial discrimination did not (Williams et al., 2017).

Cultural Issues

The experience of obsessive-compulsive disorder in different cultures appears to be relatively similar (Horwath & Weissman, 2000; Nicolini et al., 2017). The presentation of symptoms is not identical world-wide, though. In some countries, a larger number of individuals experience only obsessions and in other countries, a larger number of individuals experience only compulsions (Horwath & Weissman, 2000). Still, few studies have examined differences in the presentation of obsessive-compulsive disorder world-wide, and studies that have examined differences have utilized relatively small samples, making it difficult to draw conclusions from the data.

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